

PRE-REFERRAL STABILIZATION & REFERRAL CHECKLIST



Topic 10

PRE-REFERRAL PREPAREDNESS AND CARE DURING TRANSPORT (ALL OBSTETRIC EMERGENCIES)



1. PRE-REFERRAL CLINICAL ASSESSMENT

- ☐ Blood Pressure recorded
- ☐ Pulse recorded
- ☐ Respiratory Rate recorded
- ☐ Temperature recorded
- ☐ Oxygen saturation (if available)



2. PRE-REFERRAL STABILIZATION

- ☐ Two wide-bore IV cannulas inserted (16 G & 18 G)
- ☐ IV fluids started
- ☐ Blood sample sent (if feasible)
- ☐ Urine output monitored
- ☐ Oxygen started (if indicated)



3. PRE-REFERRAL PREPAREDNESS

- ☐ Referral slip completed
- ☐ Treatment provided documented
- ☐ Receiving facility informed
- ☐ Ambulance arranged
- ☐ Family counselled
- ☐ Trained health worker accompanies patient
- ☐ **Communicate to referral facility**

+ REFERRAL SLIP	
Name:	_____
Age:	_____
Diagnosis:	_____
Treatment given:	_____
Time:	_____
Referred to:	_____
Date/Time:	_____
Sign:	_____



4. CARE DURING TRANSPORT

- ☐ Continue monitoring
- ☐ Continue IV fluids/medications
- ☐ Position mother safely
- ☐ Maintain communication with referral facility



EARLY PREPARATION AND SAFE TRANSPORT SAVE LIVES.

STABILIZE BEFORE YOU TRANSPORT.

MONITOR, CARE AND COMMUNICATE.

PRE-REFERRAL STABILIZATION FOR POSTPARTUM HEMORRHAGE (PPH)

IMMEDIATE ACTIONS

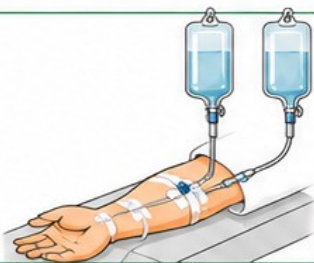


Call for help



CIRCULATION

- ☐ Two large-bore IV lines (16 G / 18 G)
- ☐ Rapid infusion of crystalloids
- ☐ Keep mother warm
- ☐ Monitor pulse and BP every 15 minutes



CONTROL BLEEDING

- ☐ Uterine massage
- ☐ Empty bladder
- ☐ Assess placenta completeness



UTEROTONICS

- ☐ Oxytocin 10 IU IM/IV
- ☐ Oxytocin infusion
- ☐ Misoprostol if Oxytocin unavailable
- ☐ Tranexamic Acid (TXA) 1 g IV



IF AVAILABLE

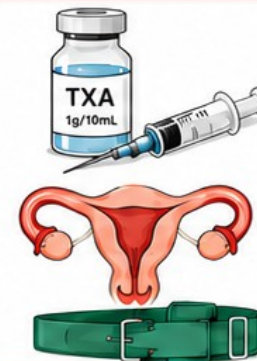
- ☐ Uterine Balloon Tamponade
- ☐ NASG application



PPH PRE-REFERRAL PREPAREDNESS (MUST BE ENSURED BEFORE REFERRAL)

HEMORRHAGE CONTROL

- ☐ Uterotonics administered
- ☐ TXA administered
- ☐ Uterine massage continued
- ☐ Balloon tamponade inserted (if indicated)
- ☐ NASG applied (if available)



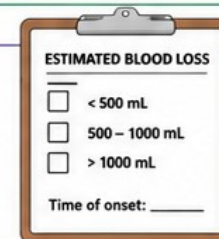
STABILIZATION

- ☐ Two IV lines functional
- ☐ IV fluids running
- ☐ Blood grouping/cross-match initiated
- ☐ Vital signs recorded



DOCUMENTATION

- ☐ Estimated blood loss documented
- ☐ Drugs administered documented
- ☐ Time of onset documented



REFERRAL

- ☐ Receiving FRU/CEmONC informed
- ☐ Blood availability confirmed
- ☐ Skilled escort accompanies patient



Never refer an actively bleeding woman without initiating treatment.

PRE-REFERRAL STABILIZATION FOR SEVERE PREECLAMPSIA & ECLAMPSIA



IDENTIFY DANGER SIGNS

- ☐ BP $\geq$ 160/110 mmHg
- ☐ Severe headache
- ☐ Visual disturbance
- ☐ Epigastric pain
- ☐ Convulsions

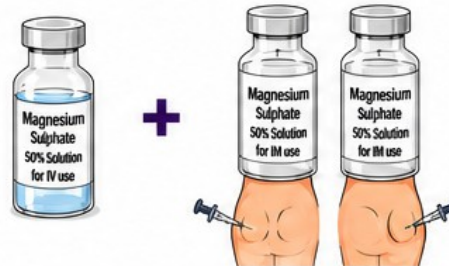


STABILIZATION

MAGNESIUM SULPHATE (LOADING DOSE)

PRITCHARD REGIMEN

- ☐ 4 g IV slowly
- PLUS**
- ☐ 10 g IM (5 g in each buttock)



CONTROL BLOOD PRESSURE



Injection
Labetalol

OR



Nifedipine
10 mg



SUPPORTIVE CARE

- ☐ Maintain airway
- ☐ Left lateral position
- ☐ Oxygen if available
- ☐ Monitor reflexes, respiration and urine output



PRE-REFERRAL PREPAREDNESS FOR SEVERE PREECLAMPSIA/ECLAMPSIA



BEFORE REFERRAL ENSURE

- ☐ Magnesium sulphate loading dose completed
- ☐ Antihypertensive administered
- ☐ BP documented
- ☐ Airway maintained
- ☐ Oxygen provided if needed
- ☐ Convulsions controlled
- ☐ Urine output assessed
- ☐ Ensure communication to referral facility



DURING TRANSPORT

- ☐ Left lateral position
- ☐ Continue monitoring BP and pulse
- ☐ Emergency drugs accompany patient
- ☐ Referral note attached
- ☐ Skilled provider accompanies patient



RED FLAGS REQUIRING IMMEDIATE REFERRAL



Recurrent
seizures



Uncontrolled
hypertension



Pulmonary
edema



Altered
consciousness



Severe fetal
distress



KEY MESSAGE

Magnesium sulphate loading dose must be administered before referral whenever eclampsia or severe preeclampsia is suspected.



REFERRAL IN REGISTER

[illegible]

REFERRAL OUT REGISTER

[illegible]

Name : PW/Mother/ Child : _____ MCTS/RCH No. : _____

Referral Institution : _____ Date of Referral : _____

Name : PW/Mother/ Child : _____ MCTS/RCH No. : _____

Referral Institution : _____ Date of Referral : _____

REFERRAL SLIP (Sub Centre)

Name of the Sub Centre: _____

Name of PW/Mother/Child: _____

Wife/Son/Daughter of: _____

Name of the Village: _____

Full Address with P.S: _____

MCTS/RCH No. : _____

Whether PW/Mother/Child is Red Card holder : _____ Yes/No _____

If yes Red card No. : _____

Date & Time of Referral: _____

Reason for Referral: _____

Investigation status (before at the time of referral)	Treatment given (before referral at the time of referral)
☒ Blood group :	1. _____
☒ Hb:	2. _____
☒ Urine Sugar/Albumin :	3. _____
☒ Others	4. _____

Management during transport: _____

Condition at time of referral: _____

Consciousness: _____ Temp: _____ Pulse: _____ BP: _____

Others (specify): _____

Information on referral provided to the institution referred to: _____ Yes/No _____

If Yes, then name of the person contacted/spoken to: _____

Facility to where patient is referred: _____

Mode of transport for referral: Govt. /PPP/ Vehicle arranged by patient: _____

Signature of HW(F): _____

Inter Facility Referral Slip

Name of the referring facility: _____

Address: _____

Name of the patient: _____, Age: _____, sex: _____,

Father's/Husband's Name: _____

Address with P.S: _____, Contact No: _____

Referred on ____/____/____ (dd/mm/yy) at _____ (time) to

_____ Name of the facility,

to where the patient is referred.

MCTS/RCH No.: (If available) _____

Provisional Diagnosis:

Admitted in the referring facility on ____/____/____ (dd/mm/yr) at _____ (time)

with chief complaints of: _____

Summary of management (Procedures, Critical interventions, drugs given for management):

Blood group :

☒ Hb:

☒ Urine Sugar&/or Albumin :

☒ Others

Condition at time of referral:

Consciousness: _____ Temp: _____ Pulse: _____ BP: _____

Others (specify): _____

Information on referral provided to the institution referred to: (Yes/No) _____

If Yes, then name of the person contacted/ spoken to: _____

Mode of transport for referral: Govt. /PPP/ Vehicle arranged by patient: _____

signature of Referring Physician/Health functionaries
(Name/Designation/Stamp)

Referral Checklist for Obstetric Cases

Use this checklist before referring to any obstetric emergency patient.

Patient Information

Name: _____ Age: _____

Diagnosis/Indication for Referral: _____

Referring Facility: _____

Receiving Facility: _____

Date & Time of Referral: _____ Date & Time of arrival: _____

Accompanying Health Worker: _____

Will be shared

S.L. No	Activity	Yes	No	Not applicable
1. Pre-Referral Clinical Assessment				
1.1	Blood Pressure recorded			
1.2	Pulse recorded			
1.3	Respiratory Rate recorded			
1.4	Temperature recorded			
1.5	Oxygen saturation recorded (if available)			
2. Pre-Referral Stabilization				
2.1	Two wide-bore IV cannulas inserted (If in shock)			
2.2	IV fluids started			
2.3	Blood sample sent (if feasible)			
2.4	Urinary catheter inserted (If applicable) and time of insertion			
2.5	Oxygen started (if indicated)			
3. Pre-Referral Preparedness				
3.1	Referral slip completed			
3.2	Treatment provided documented			
3.3	Receiving facility informed			
3.4	Ambulance arranged			
3.5	Family counselled			
3.6	Referral facility is communicated			
3.7	Trained health worker accompanies patient			
4. Care During Transport (At Ambulance)				
4.1	Continue monitoring			
4.2	Continue IV fluids/medications			
4.3	Position mother safely			
4.4	Maintain communication with referral facility			

Signature & Seal of referring doctor: _____

Name of Ambulance attendant: _____

Signature of ambulance attendant: _____



CHILD HEALTH

